Effective

Celsite® extra small size: Clinical Strategy

Title	Name
Deputy Director in charge of Quality and delegated Regulatory Affairs [Author]	Patrick RAUGEL
Director [Reviewer]	Sylvie PLUMET

INTRODUCTION

The project to develop an extra-small chamber to complement the Celsite range of access ports involves providing sufficient clinical evidence to confirm the conformity of the device thus created with relevant general safety and performance requirements (annex I of MDR 2017/745 UE) under the normal conditions of the intended use of the device.
The clinical data will also enable the evaluation of the undesirable side-effects and the acceptability of the benefit-risk- ratio.
The goal of this document is to outline the strategy to collect, analyze and report the clinical data that will provide the expected confirmation of conformity.

CONTEXT

The device to be developed, the Celsite extra small, is intended to fill a gap at the intersection of three product ranges, Epoxy, PSU and Safety. The table 1 thereafter provides an overview of the ranges of access ports.

Effective

Document No.: PRO N°169 - Version: 1.0 - Document ID: BB-RECHA-008341 Cette édition imprimée n'est plus valide 24h après le: 2026-10-11 19:31 (CET

Effective

Table 1 : overview of the ranges of access ports

	Sizes		
Product ranges	Extra small	Small	Standard
Epoxy			
PEEK Safety OR PSU	Device to be developped	 	 

The indications for use, the users, the patients, the implantation technique and the accessories, as well as the expected clinical benefit are unchanged from existing devices. As a consequence, the scope of the clinical evaluation should remain unchanged.
The clinical scope of the device to be developed is outlined in the table 2 thereafter.

Table 2 : clinical scope of the device to be developped

Intended use	Celsite® venous access port systems are designed to allow safe repeated access to the venous system for administration of fluids and/or for blood withdrawal. Celsite® venous access port systems are sterilized using ethylene oxide, are sterile, non-pyrogenic, in unopened, undamaged, individual packaging. Celsite® venous access port systems can be placed either by percutaneous or surgical techniques. Celsite® venous access port systems may be used for Contrast Enhanced Computerized Tomography (CECT) using high pressure injection
Indications	**Celsite® venous access port systems are indicated** for patient therapies that require repeated, prolonged access to the venous system. Celsite® venous access port systems are designed to be used for injection of chemotherapies, antibiotics, anesthetics, antifungals, antinociceptives, antiviral drugs, intravenous nutrition, contrast media products (see VII-4 High pressure injection), intravenous fluids (isotonic, hypotonic, and hypertonic), blood products, blood sampling and transfusion. **Areas of therapeutic application include**: Oncology | Palliative care | Immunology/hematology | Infectiology | Radiology for diagnosis purpose | Medical biology

Effective

Document No.: PRO N°169 - Version: 1.0 - Document ID: BB-RECHA-008341 Cette édition imprimée n'est plus valide 24h après le: 2026-10-11 19:31 (CET

Contraindications	**Celsite® venous access port systems are contraindicated for patients whenever:** The presence of infection, bacteremia, or septicemia is known or suspected. | The patient has known allergy to the materials contained in the device | The medications used are incompatible with any of the materials contained in the access port system. | The patient's anatomy does not allow the insertion of the catheter and/or access port at the planned access site (vein diameter, body size, e.g., obesity). | The patient has severe chronic obstructive pulmonary disease. | The patient has or has had previous radiotherapy in the chosen area. | The patient has a bad skin condition in the chosen area (e.g., burns). | The patient has experienced previous episodes of venous thrombosis. | The patient has a severe, uncorrectable coagulopathy. | The patient has a superior vena cava syndrome (e.g. obstruction or compression of the vein due to a tumor). | The patient has a known or suspected obstruction in the vasculature (contraindication to use Guidewire). | The patient should have an axillary lymph node dissection (e.g. breast cancer) (contraindications for brachial implantation)
Intended user population	**Celsite® venous access port systems are intended for use by** healthcare practitioners who must have experience with the technical and clinical requirements of access port implantation, use, complications, maintenance and retrieval. The access port must be used and maintained by experienced healthcare practitioners qualified in the use of implantable access ports.
Intended patient population	Celsite® venous access port systems (extra small, small and standard sizes) can be implanted in both pediatric patients/children and adults with a body mass superior or equal to 10Kg, regardless of age or ethnic origin who require repeated and long-term venous access. Exception is for extra-small ports (baby-brachial 5F PUR and babyport ® 4.5F PUR / 6F SI) which could be implanted in paediatric population with a body mass superior or equal to 3,5Kg. Limitation in terms of body mass is due to the ethylene oxide residue.
Safety and clinical performance	The clinical performance of Celsite® venous access port systems is to deliver a fluid in the venous system. The clinical benefit of Celsite® venous access port systems is to decrease the risk of infection compared to external catheters.
Potential complications	**Potential complications in relation to implantation, use, maintenance, and removal techniques** Air embolism | Artery or vein damage/injury | Arteriovenous fistula | Accidental blood exposure | Bleeding | Brachial plexus injury | Bruising | Cardiac arrhythmia, cardiac tamponade, cardiac perforation | Endocarditis | Formation of deposits | Hematoma | Hemothorax/hydrothorax | Hemorrhage | Pneumothorax | Pleural effusion | Nerve damage | Infection/sepsis/bacteremia | Risks associated with anesthesia, surgery, post-operative recovery | Risks associated with contrast media injection | Swelling
	Potential complications in relation to the implanted device Catheter disconnection, rupture or fragmentation | Catheter damage or breakage due to Pinch-Off | Syndrome (see II-6-3 Pinch-Off Syndrome) | Catheter migration / catheter embolization | Clot formation | Death | Drug extravasation | Fibrin sheath formation | Implant rejection/intolerance reaction including allergic reaction | Inflammatory reaction | Intravascular catheter encapsulation | Pain | Vessel perforation/laceration/erosion | Pericarditis | Port/catheter occlusion | Port reversal | Retraction or malposition of the catheter (intravenous, cardiac, arterial) | Tenderness over the area of the access port Thoracic duct injury | Thromboembolism/thrombophlebitis | Thrombosis | Twiddler syndrome | Venous stenosis | Wound dehiscence including necrosis or scarring of the skin over the access port

STRATEGY

Collect, analyze and report general clinical data – state of the art

The initial state of the art needs to be updated, without changing its scope, as the medical device to be developed does not aim to bring any significant changes to the treatment approach.
The update will mainly focus on collecting and analyzing the most recent publications.

Collect, analyze and report device-specific clinical data – PMCF study

Clinical data specifically concerning the portfolio of access ports is being collected as part of a PMCF study. This data, which is partially available to date, will continue to be collected during the development of the extra small chamber and will be analyzed with regard to the performance and safety of the different ranges and models from the portfolio.
A segmented analysis of the data will allow particular attention to be paid to access ports of comparable size.

Establish a clinical evaluation plan
A clinical evaluation plan as referred to in Article 61(1) of the MDR and established in accordance with the specifications of Annex XIV, point 1.a, shall include the device to be developed. The clinical evaluation plan will be established in accordance with instruction IMS-SOP-AA 4-1-07.

When the new device is launched on the market, the other devices in the existing ranges will be CE marked under the MDR.

Due to the design logic that is an extension of range (see Table 1, for reference), it is planned to perform an equivalence analysis which will show, all other things being equal, that:

- The materials used and the manufacturing processes for the device to be designed will be equivalent to the materials used and the manufacturing processes for an existing Safety chamber or PSU chamber;
- The size of the chamber, outlets, and catheters are equivalent to those of a brachial epoxy chamber.

The equivalence analysis will cover technical, clinical, and biological aspects. All the details of each domain will be compared and documented.

In addition, the need for a specific PMCF study focusing on the newly added device will be evaluated.

Produce a clinical evaluation report
A clinical evaluation report will be prepared in accordance with the provisions set out in the clinical evaluation plan. This report will be prepared in accordance with clinical evaluation instruction IMS-SOP-AA 4-1-07.

Evaluate continuously the relance of the strategy – Revise when necessary

The current strategy may require revision and adaptation if any of the assumptions made during its drafting change. This may be necessary in the following cases in particular:

- Change affecting equivalency
  Any change that could lead to a break in equivalence between existing ranges and the system to be developed could call into question the chosen approach.

- Change related to the design or manufacture of the device:
  - Material
  - Indication,
  - Implantation procedure
  - accessories

- Change related to the intended use, user, patient or clinical benefit
- Change related to the current knowledge of clinical practices or outcomes
  - Discovery of unknown complications
  - Discovery of a new therapeutic trend highlighted by the state of the art

Title: Celsite extra small size - Clinical Strategy - PRO169 Initiator: Christine _ Gaboriaud

This document is signed electronically in compliance with the B. Braun electronic signature policies and procedures by following persons:

UserName: Gaboriaud, Christine (gabocrfr)
Title: SA-FR02-M-Quality Assistant
Date: Wednesday, 17 December 2025, 17:56 W. Europe Daylight Time
Meaning: Document signed as Author
==========

UserName: Raugel, Patrick (raugpa16)
Title: SA-FR02-M-RAQA Director
Date: Thursday, 18 December 2025, 08:33 W. Europe Daylight Time
Meaning: Approve Document
==========

UserName: Plumet, Sylvie (plumsyfr)
Title: SA-FR02-M- Coe Director
Date: Monday, 22 December 2025, 09:11 W. Europe Daylight Time
Meaning: Approve Document
==========